ABSTRACT SUBMISSION

Abstract submission guidelines

1. Eligibility and submission

The Scientific Program Committee invites original research abstracts spanning chemistry, pharmacology, biotechnology, and translational drug discovery, with a focus on neuro-metabolic disorders, cancer, and safety sciences. Submissions are open to faculty members, scientists, postdoctoral researchers, PhD scholars, and postgraduate students (MD/MS/M.Pharm/Pharm.D/M.Sc) from recognized academic and research institutions across India and internationally. Registration is required before submission. Verify your receipt Registration ID and registration email in the portal. The abstract body must not exceed 300 words, excluding the title, authors and affiliations.

2. Structured format — maximum 300 words

Title	Brief, informative and in Title Case. Avoid abbreviations.
Authors & affiliations	Give the full names of all authors, their departments, institutes, city, state and PIN code. Underline the presenting author. Mark the corresponding author with an asterisk (*) and provide a valid email address.
Background & rationale	Explain the scientific gap, unmet therapeutic need and primary hypothesis in two or three sentences.
Materials & methods	Describe the study design, models or cell lines, agents and dosages, assays, instrumentation or in-silico methods. Include the IAEC/IEC ethics-clearance number where applicable.
Results & discussion	Summarize primary quantitative findings and statistical significance (for example, $p < 0.05$; mean ± SEM). Do not include figures, graphs or tables in the abstract text.
Conclusions & impact	State the main conclusion and its translational relevance to drug development or clinical therapeutics.
Keywords	Provide four to six relevant keywords, separated by semicolons.

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Scientific tracks and key dates

3. Choose one thematic track

Select the most appropriate track in the online abstract-submission form.

TR-01
Neuropharmacology & Neurotherapeutics

TR-02
Cerebrovascular & Neuro-Metabolic Pharmacology

TR-03
Cancer Pharmacology & Neuro-Oncology

TR-04
Biotechnology & Translational Disease Models

TR-05
Medicinal Chemistry & Target Validation

TR-06
Synthetic Chemistry & Chemical Biology

TR-07
In-silico & AI-Driven Drug Design

TR-08
Pharmacovigilance-Guided Drug Repurposing & Network Pharmacology

TR-09
Indian Indigenous Systems & Ethnopharmacology

TR-10
Toxicology & Safety Pharmacology

4. Important dates

Abstract deadline	31 December 2026
Acceptance notice	15 January 2027

5. Before you submit

Prepare a Word document (.doc or .docx), no larger than 1 MB, using Times New Roman or Calibri, 12 pt, with 1.15 line spacing. Check the word limit, author details, ethics information and selected track. Register for the conference first, then verify your Registration ID and the same registration email in the submission portal.

Submit online
https://neuro-med-pharmacon-27.vercel.app/#abstracts

AUTHOR TEMPLATE · MAXIMUM 300 WORDS

Structured abstract example

Illustrative example only. Replace all example names, study details, ethics approvals and findings with your own research.

Molecular Dissection of Neuroinflammatory Signaling in Ischemic Stroke

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Track: TR-01 — Neuropharmacology & Neurotherapeutics

Background & objectives

Ischemic cerebral stroke initiates acute microglial polarization accompanied by catastrophic neurovascular breakdown. Despite recanalization therapies, post-ischemic secondary neuroinflammation remains therapeutically unaddressed. Here, we hypothesized that pharmacological inhibition of NLRP3 inflammasome priming via a novel small-molecule scaffold suppresses neurotoxic cytokines and salvages ischemic penumbral tissue.

Materials & methods

Adult male Sprague-Dawley rats (250–280 g, IAEC Approval # NIPER/IAEC/2026/04) were subjected to 90-minute transient middle cerebral artery occlusion (tMCAO) followed by 24h reperfusion. Vehicle or Test Agent (5, 10, 20 mg/kg i.v.) was administered at reperfusion onset. Neurological deficit scoring (mNSS), TTC cerebral infarct planimetry, blood-brain barrier permeability (Evans blue extravasation), and immunofluorescence for Iba-1/GFAP were quantified.

Results & discussion

Test Agent (10 mg/kg) caused a 48.6% reduction in total cerebral infarct volume compared to vehicle-treated controls ($p < 0.001$, one-way ANOVA with Dunnett's post-test). Neurological deficit scores improved significantly within 24 hours (mNSS 5.2 vs 11.8 in vehicle). Western blotting of penumbral tissue confirmed downregulation of cleaved caspase-1 and mature IL-1beta without altering systemic hemodynamics.

Conclusions

The findings establish this novel pharmacophore as a potent, brain-penetrant neurovascular protective lead candidate capable of dampening neuroinflammation during the hyperacute reperfusion window.

Keywords: Ischemic stroke; NLRP3 inflammasome; Microglia; Neuroprotection; NIPER-Raebareli

Presentation preference: [ ] Oral presentation [ ] Poster presentation